

Table 5: Quantiles of potential outcomes and quantile treatment effects (grams)

(grams)		$Q_{0.15}$	$Q_{0.25}$	$Q_{0.5}$	$Q_{0.75}$	$Q_{0.85}$
Entire Sample	QTE	195	214	234	259	292
	$Q(Y_0)$	2760	2927	3220	3515	3675
	$Q(Y_1)$	2955	3141	3454	3774	3967
White	QTE	204	212	212	227	255
	$Q(Y_0)$	2815	2974	3300	3589	3731
	$Q(Y_1)$	3019	3186	3512	3816	3986
SCCG	QTE	109	165	187	244	194
	$Q(Y_0)$	2908	3031	3316	3566	3798
	$Q(Y_1)$	3017	3196	3503	3810	3992
Age 26-35	QTE	233	180	179	262	283
	$Q(Y_0)$	2781	3008	3331	3557	3720
	$Q(Y_1)$	3014	3188	3510	3818	4003

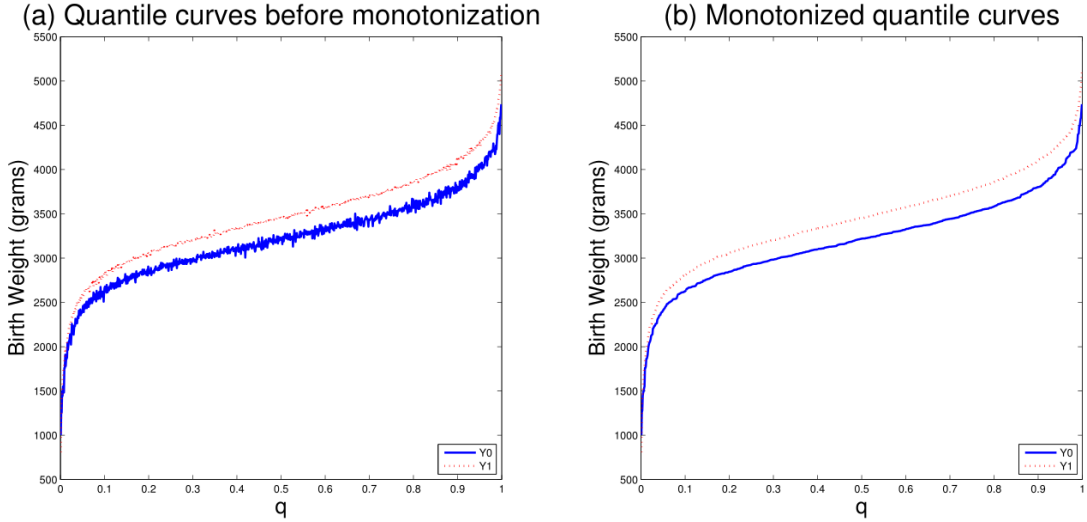


Figure 16: Estimated quantile curves

the monotonized quantile curves for Y_0 and Y_1 , respectively. The marginal distribution functions of Y_0 and Y_1 are obtained by inverting the monotonized quantile curves.

Table 5 presents estimates of quantiles for potential outcomes and QTE. One noticeable observation is that for SCCG women, low quantiles ($q < 0.5$) of birth weight from smokers are remarkably higher compared to those for the entire sample or other subgroups, while their nonsmokers' birth weight quantiles are similar to those in other groups. This leads to the lower quantile smoking effects for this college education group compared to other groups at low quantiles.

I also obtain the proportion of potential low birth weight infants to smokers and nonsmokers, $F_0(2, 500)$ and $F_1(2, 500)$, respectively. As shown in Table 6, 6.5% of babies to smokers would have low birth weight,